Follow-up Continuity in a Clinic

Buyer-hypothesis discussion draft

FICTIONAL / UNVERIFIED - exercise material only
No real clinic, staff, patient data, research, buyer validation, or outcomes are represented.

The question-not a conclusion

Hypothesis: Clinic staff may lose follow-up continuity between inquiry, booking, and visit.

We do not yet know whether this happens, why it happens, how often it happens, or whether an added tool would help. First test workflow ownership, fit, and escalation needs with clinic staff.

Synthetic exercise figures

SYNTHETIC ONLY - not clinic performance or evidence

same synthetic reporting period	Exercise figure
Inquiries	86
Booked	62
Visited	41
Associated revenue	USD 16400

These figures have no supplied source records or definitions. They do not establish conversion, causation, revenue impact, or product effectiveness.

Map the current workflow first

Inquiry → booking → visit

- Who owns each handoff today?
- Which tools and reminders are already used?
- Where, if anywhere, does ownership become unclear?
- What works reliably-and what would disprove the hypothesis?

Use fictional examples. Do not collect patient-identifying information.

A possible direction, not a promised solution

Explore whether a lightweight **administrative** follow-up aid could fit existing work.

- Do not duplicate useful existing steps.
- Keep staff in control of review and action.
- Do not automate clinical judgement or give clinical advice.
- Treat privacy, accessibility, records, and integration as requirements to investigate-not capabilities claimed here.

Staff review is required

Proposed design principle

Every administrative follow-up suggestion or queue item is reviewed by an authorized staff member before action.

Research questions: Who is authorized? What must they verify? How is an item corrected, dismissed, or documented? What happens during workload peaks or staff absence?

No automated patient communication or workflow change is authorized by this draft.

Clinical escalation is explicit

Administrative work must stop at the clinical boundary

- Staff and a qualified clinic representative define escalation triggers and urgency.
- Route clinical concerns to the clinic-designated clinical role; do not have a tool decide or answer them.
- Agree on a fallback when the designated person is unavailable.
- Keep escalation rules clinic-approved; no thresholds are invented here.

Unaided pricing questions

Ask before showing a price, package, or synthetic metric.

> "What, if anything, would you pay for this in your current situation? Please state an amount and currency, including zero if that is your answer."

> "Who would approve that spend, from which budget, and what would need to be true?"

Zero / USD 0 are valid answers. Record the exact unprompted amount and currency. Do not anchor on synthetic associated revenue.

Research before any pilot decision

1. Interview relevant staff and map handoffs using fictional examples.
2. Test both supporting and disconfirming evidence; request only safe, voluntary aggregate context.
3. Confirm workflow fit, accountable owner, staff-review steps, and clinic-defined clinical escalation.
4. Ask unaided pricing questions and identify the budget decision-maker.
5. Decide whether to continue discovery; a pilot needs separate approval and safeguards.

Current status: evidence unavailable; no buyer validation; no pilot recommendation.